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5 September 2026

Ms Noelle Cridland
Chief Executive
Metro South Hospital and Health Service

By email. Copy to Ms Jacqui Roberts and Ms Michelle Harrison, Injury Management Consultant, Logan and Beaudesert Health Service.

**Determination of my application for special leave on full pay, and related matters —
MSH-INJ-5795**

Dear Ms Cridland,

I write to you because my application for special leave on full pay, made on 28 July 2026 and renewed on 5 August 2026, has not been determined, and because clause 6.1 of Directive 12/24 places the power to approve such leave with the chief executive. The other matters below bear on that decision, and one of them concerns a Deed of Settlement to which you and I are the parties.

1. THE POSITION

I am absent from the workplace at the Health Service's direction. I have held an Employee Capability Checklist since 3 July 2026 certifying me fit for my role with restrictions, and I have remained available to work under the arrangement the Health Service itself approved on 9 June 2026. The Health Service has said it requires clarification of my medical position. It has held my signed authority to obtain that clarification directly from my treating psychiatrist since 24 August 2026. My psychiatrist put four questions to Injury Management on 19 August 2026, and confirmed to me on 5 September 2026 that he is still waiting for a response before he provides the report.

I have received no wages for work since 13 July 2026. Throughout that period I have funded the absence myself. My paid sick leave was exhausted, then my recreation leave, then half pay, and the absence has since been recorded as leave without pay.

A payment was made to me on 4 September 2026. It followed a series of requests, each of which is documented:

3 July 2026, 3:18 pm	that the balance be processed as annual leave rather than sick leave without pay
10 July 2026, 8:35 am	that the shifts be treated as paid time and not deducted from my leave, and that my return be facilitated — unanswered
13 July 2026, 4:39 pm	the same request again; the basis in writing if refused; interim duties offered
22 July 2026	Payroll Enquiry 4438861 lodged
28 July 2026, 5:39 pm	special leave on full pay applied for
30 July 2026, 11:25 am	that the shifts from 3 July be processed by ad hoc payment and the leave re-credited
3 August 2026, 11:38 am	Stage 1 notice of dispute under EB12 cl 1.11, covering leave and pay
5 August 2026, 7:30 am	that my recreation leave not be applied, and that the period be coded as special leave on full pay
31 August 2026, 10:59 am	formal application for long service leave on full pay from 13 July 2026, with annual leave on its exhaustion
2 September 2026, 11:07 am	ad hoc payment requested to be processed that day
3 September 2026, 1:30 pm	the coding applied since 3 July, and payment under HR Policy C13

The payment made on 4 September 2026 was of my own leave entitlements. It was not wages. I am now told that I must wait until 29 September 2026 to fund the absence from seven years of service.

I do not accept that I should be funding this absence at all. Every application I have made for leave has been made under protest and subject to re-credit, and this letter is written on the same basis.

2. THE MEDICAL INFORMATION

On 13 August 2026 Dr Krishnaiah wrote to Injury Management. Ms Harrison replied the same afternoon, acknowledged that I had given permission, and wrote: "Happy to provide you with this information and to assist you with this." She recorded the reason for the request as being that I "had requested a reduction in hours and advised this request was due to medical reasons".

On 19 August 2026 Dr Krishnaiah put four questions to Injury Management. He asked on what basis my earlier reductions in hours were accepted without medical certification; what has changed in the administrative position that warrants an in-depth psychiatric assessment; what the policy is on disclosure of sensitive mental health information and the grounds for it; and what concerns or risks observed or reported at the workplace warrant disclosure. He wrote that this "will help before I undertake detailed assessment to provide my professional opinion".

Those questions have not been answered. The correspondence is enclosed.

I am not asking the Health Service to answer to me. I am asking that it answer the clinician, so that the report it says it requires can be produced.

3. THE APPLICATION FOR SPECIAL LEAVE, WHICH REMAINS UNDETERMINED

I applied for special leave on full pay on 28 July 2026, and again on 5 August 2026. Neither application has been determined. I ask that you determine it.

Clause 9.12.1 of EB12 provides that Directive 12/24: Special Leave applies to all employees covered by the Agreement. Clause 6.1 of that Directive provides that "a chief executive may approve paid leave for employees for any purpose, with duration appropriate to the purpose of the leave", and clause 6.1(a) provides that leave may exceed five working days per year per reason "where the chief executive considers that circumstances warrant the granting of additional paid leave", such leave being "reasonable and proportionate to the circumstances".

Clause 6.5 requires that in determining an application under clause 6.1 a chief executive must consider the reason the leave is requested, its duration, and the impact on the employee if it is not approved. On each of those:

The reason. I am absent from the workplace at the direction of the Health Service, pending medical information the Health Service has sought and has not yet obtained. I am certified fit for my role with restrictions and I have remained available to work.

The duration. I ask for a period ending on the earlier of the conclusion of the medical process or the date my long service leave becomes available. The request is bounded, and one of the two end points is within the Health Service's own control.

The impact if it is not approved. I have had no wages for work since 13 July 2026, and I have lost my housing. Beyond that, since 3 July 2026 I have made two applications for special leave, an application for long service leave and a Stage 1 dispute notification, and I

have corresponded separately with my line manager, Payroll Services, Injury Management and Human Resources on each of them. Each has required me to identify the instrument, make the application, follow it up and, in most cases, follow it up again. The Stage 1 notification lapsed without outcome. On 28 August 2026 I withdrew the Stage 2 referral, so that these matters could be worked through collaboratively, expressly reserving the questions of pay, the leave debited since 3 July 2026 and the adjustments identified in the Employee Capability Checklist as unresolved and in dispute. I have not renewed that referral. I have done all of this without income, and while certified fit and available to work. I ask that the burden of conducting these matters at arm's length, over this period and in these circumstances, be weighed as part of the impact under clause 6.5(d).

I ask that this application be determined whichever course is taken under paragraph 4 below. Even if Dr Krishnaiah's questions were answered today, his report, its consideration and any return to work would take further time, and I have no income during that time. The determination of the special leave application does not depend on the medical process concluding; it is what makes the medical process survivable.

If the power under clause 6.1 has been delegated, could you please tell me to whom, and by when the application will be determined. In the five weeks since it was made, no decision-maker has been identified to me.

If the application is refused, I ask for the decision and the reasons in writing.

4. WHAT WOULD BRING THE ABSENCE TO AN END

Separately, and in addition, I ask that you decide one of the following and tell me which:

- (a) that Dr Krishnaiah's questions of 19 August 2026 be answered, so that the report the Health Service says it requires can be produced; or
- (b) that I return to work under the arrangement approved on 9 June 2026.

Either would end the absence. Neither, by itself, addresses the period already elapsed or the period until a return takes effect, which is what paragraph 3 concerns.

5. LONG SERVICE LEAVE

This is a separate matter, and I put it differently, because it is an entitlement rather than a request.

Clause 22(c) of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides that “employees who have completed 7 years’ continuous service are entitled to take long service leave on full pay or half pay”. Clause 8.1 of Directive 10/24 provides that “employees are entitled to take pro rata long service leave after 7 years continuous service”. Section 1 of HR Policy C38 is to the same effect.

Recital B of the Deed records that my employment commenced on 25 March 2019. Seven years of continuous service therefore fell on 25 March 2026.

On 3 September 2026 Payroll advised an eligibility date of 4 September 2026. On 4 September 2026 I was advised of 29 September 2026. The difference between 25 March 2026 and 29 September 2026 is 188 days, and it is made up of three periods recorded as leave without pay — 8 May to 19 June 2022, 21 June to 20 September 2024, and 13 December 2024 to 23 February 2025.

Could you please:

1. identify the provision under which periods of approved leave without pay are excluded from continuous service for long service leave purposes. I have read the *Industrial Relations Act 2016*, the Award, EB12, HR Policy C38 and Directive 10/24, and I have not been able to identify it;
2. tell me on what basis access at seven years is refused; and
3. determine my application of 31 August 2026. Clause 7.4 of Directive 10/24 requires the employer to respond to a request to take long service leave in a timely manner, indicating whether the leave applied for has been approved or not.

I ask questions 1 and 2 while maintaining that I should not be funding this absence from my own entitlements at all. The two matters are separate: whether the eligibility date is correct, and whether I should be required to reach it.

The calculation appears to have been made by reference to leave codes alone. It does not appear to take account of the Deed, under which I was reinstated with an effective date of 20 September 2024, and by which the Health Service reserved the matters of my absence rather than determining them. I ask that the calculation be reviewed with the Deed before the reviewer.

6. THE DEED

Clause 6 of the Deed reserved to the Health Service the right to deal with the outstanding matters of my absence. Clause 7 reserves my rights in relation to any future management action taken pursuant to clause 6.

Could you please tell me whether the process commenced on 3 July 2026 is management action under clause 6 of the Deed.

7. ENCLOSURES

1. Dr Krishnaiah's email of 27 August 2026, forwarding his correspondence with Injury Management of 13 and 19 August 2026 (9 pages)
2. Limited Scope Authorisation for Disclosure of Medical Information, signed 24 August 2026
3. Dr Krishnaiah's email of 5 September 2026
4. Schedule of the provisions relied on (one page). I have not enclosed the instruments themselves, which the Health Service holds.

I remain certified fit for my role with restrictions, and available to work. I would be grateful for a decision.

Kind regards

Cory Shepherd

Employee number 388372